

--

Name: إيمان محي الدين عبدالحميد الجمال National ID: 28309011700345	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

DMS
زولاد کس

جمهورية مصر العربية
بطاقة هوية شخصية

أحمد
مكي الدين صلاح الدين الجمل
مستشفى كامل النوري - البحر الشرقي
شبين الكوم - الشرقية

١٩٨٣/٠٨/٠١ ٢٨٢٠٩٠١١٧٠٠٣٤٥

ID8897582

٢٠١٩/٠٩ ٢٨٢٠٩٠١١٧٠٠٣٤٥

حاصلة على بطاقتها
لمدة عامين
إلى
معدن جلال صلاح الدين الجمل
الطبعة السادسة هي ٢٠٢٣/٠٩/١٧

١٢



A-Patient ID:

1- Name : اسم المريض / اسم العائلة

Sex:

3- Medical numbers:
Female

83038646

Old Med:

598/2022

2- ID numbers: 28309011700345

Patient Age: 40 Y

Family history

Marital status

Co morbidity

Phone

01004283552

BMI

NRS Score

GROUP

Menopausal
status

PREMENOPAUSAL

REASON OF I

Date of MDT

Breast Imaging:

Clinical data

Mets Workup:

BX:

IDC

GII

ER:

7/8

PR:

8/8

Her 2 Neu:

0

SISH:

T

N

M

Sonomammography

PATIENT HISTORY

PT RECEIVED NACTH-->ACX4-->TAXOL X12 WK

HAD BCS+SLN-->CR,ON FEMARA

DEXA SCAN-->OSTEOPENIA

MDT OUTCOME & PLAN

FOR ZOMETA EVERY 6 MONTHS

MEDICAL ONCOLOGY

SURGERY

Psychiatric

SIGNATURE

RADIOLOGY

PATHOLOGY

Nutrition

Name: إيمان محي الدين عبد الحميد		
MR. : 5377		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient presented for a tamoxifen follow-up; a transvaginal ultrasound performed on 19-01-2026 demonstrated an endometrial thickness of 5 mm, and a thyroid ultrasound on the same date was reported as normal. The patient is a 42-year-old, premenopausal individual with right-sided breast cancer, clinical stage T2 N0 M0. Neoadjuvant chemotherapy was administered, followed by breast-conserving surgery with sentinel lymph-node biopsy in July 2022. Post-operative radiotherapy was completed, and endocrine therapy was initiated in August 2022. The current regimen includes tamoxifen and Zoladex; prior therapy with letrozole was discontinued because of elevated estradiol levels. A recent DEXA scan revealed osteopenia, and the patient received zoledronic acid today in addition to tamoxifen and Zoladex, with laboratory monitoring planned for three months. Neck ultrasound showed a solitary TIRADS 3 nodule. Contrast-enhanced CT of the chest, abdomen, and pelvis demonstrated resolution of previously noted supraclavicular and mediastinal lymph nodes, with no residual disease. A bone scan performed in July 2026 was negative. The patient will continue tamoxifen and Zoladex as ongoing therapy.		
تحريراً في ,		
	التاريخ : 18-09-2026	الوقت :
Reham Ahmed Fathy Ibrahim		الطبيب المعالج :
يعتمد ,		

Patient Name: Eman Mohye Al Din Abdel Hameed Al Gamal	Date : 03/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 598/2022	Pat. ID: 15282

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Post right CBS and ALAND followed by radiotherapy, follow up study.

Findings:

Right breast:

- Scattered fibroglandular densities. (ACR b)
- Upper central posterior third architectural distortion with no speculations, n asymmetry , or suspicious microcalcifications.
- Thickened skin and coarsen trabeculae.
- Ultrasound demonstrated upper central posterior third, area of parenchym disruption underneath the operative scar with no solid , cystic lesions or flu collections.
- Thickened skin reaching 4.5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast:

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses , distortion, asymmetry , or sus microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions .
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 2): Post-therapeutic sequelae

Left breast (BIRADS 1): Normal study.

Recommendation:-

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast

assessment combined with recommendations:

Category & recommendations

Suspected Cancer Risk

BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	



Patient Name: Eman Mohye Al Din Abdel Hameed Al Gamal	Date: 03/08/2025
Exam: CT CAP(CT Chest Abd. Pelvis)	Age: 42 Year(s)
Medical ID: 598/2022	Pat. ID: 15282

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN

A case of breast cancer

Findings:

I Chest:-

- Left breast architectural distortion with skin thickening .
- Right upper lobe subpleural reticulation ?post radiotherapy.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Few prominent slightly dense mediastinal lymph nodes , the largest at subacral node 6mm ?? post granulomatous sequel.
- Left supraclavicular L. N 9 x6 mm.
- Few tiny pulmonary nodules average mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of mild enlarged in size with homogenous CT texture. No definite focal lesions (contrast basis) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Left upper ureteric non obstructing stone 5mm.
- Left midcalyx stone 3mm.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Multiple pelvic phelolith seen.
- IUCD in place

III Scanned skeleton:

- No worrisome lytic or sclerotic bony lesions

Opinion:

- No hepatic pulmonary or osseous deposits .

Dr. Reham Kfu

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology, Ass. Prof. of Pathology,
Fellow, Mass General Hospital, Harvard
Member, United States Canadian Academy of Pathology

Patient Name : ايمان محي الدين عبد الحميد

Age : 38 Y

Referred By Prof. Dr. : محمود جمال حجاج

Clinical Diagnosis : Right breast mass

Nature of Specimen : Block (Tru cul biopsy)

Receiving Date : 2022-01-22

Analysis ID : 6195

Gender : Female

Patient ID : 482474

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.

The sections were treated against:

- Estrogen receptors
- Progesterone receptors
- Her 2
- Ki67

Results :

Sections treated for Estrogen receptors revealed POSITIVE Nuclear staining by chromogen.

Sections treated for Progesterone receptors revealed POSITIVE Nuclear staining by chromogen.

Sections treated for HER2 revealed NEGATIVE CELL MEMBRANE STAINING.

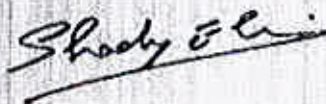
22 % of Tumour cells are POSITIVE for Ki67.

Conclusion :

Breast, Block, TUMOUR CELLS ARE

- POSITIVE FOR ESTROGEN RECEPTORS (SCORE 7 / 8)
- POSITIVE FOR PROGESTERONE RECEPTORS (SCORE 8 / 8)
- NEGATIVE FOR HER 2 PROTEIN OVEREXPRESSION, SCORE 0.
- THE PROLIFERATION INDEX OF KI67 IS 22 % OF TUMOUR CELLS.

Prof. Dr. Shady Elia Anis



Prof. Dr. Elia Anis Ishak



This report is electronically signed, any abrasion or modification would render it void.

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology, Ass. Prof. of Pathology, Ca
Fellow, Mass General Hospital, Harvard
Member, United States Canadian Academy

Patient Name : ايمان محي الدين عبد الحميد

Analysis ID : 619577

Age : 38 Y

Gender : Female

Referred By Prof. Dr. : محمود جمال حجاج

Clinical Diagnosis : Right breast mass

Nature of Specimen : Tru cut biopsy

Receiving Date : 2022-01-22

Patient ID : 482474

PATHOLOGY REPORT

Gross :

Two tan yellow tissue cores, the longest measured 1.7 cm long, totally submitted.

Microscopic :

Serial sections examined from the specimen received revealed cores of tumour tissue fo of irregular groups of malignant ductal cells, showing mild to moderate nuclear pleomorp and hyperchromasia. There is moderate attempt at tubular formation.

There are moderate number of mitotic figures.

There is marked desmoplasia.

There is peritumoural lymphocytic infiltrate around 5 %.

No tumoral necrosis.

No intraductal component.

Diagnosis :

Right breast mass, Tru cut biopsy, INFILTRATING DUCT CARCINOMA, GRADE II.
- REQUESTED HORMONAL RECEPTORS IN A SEPARATE REPORT.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia Anis

This report is electronically signed, any abrasion or modification would render it void.

هذا التقرير تم توقيعه إلكترونياً، أي تغيير أو تعديل في النص أو التوقيع سيؤدي إلى بطلان التقرير.

Patient Name : الأستاذ / ايمن محي الدين عبدالحميد الجمل	Request No. : 13008471
Patient No. : 28309011700345	Request Date : 20/07/2022
Path No. : 1976/22	Verification Date : 15/08/2022
Sex : Female	Financial Category : نفقة الدولة
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : marwa moham

Pathology Report

Clinical Data:

Right breast cancer , post NACT

Nature of the specimen:

BCS & SLNs

Gross Examination:

A sector of mammary pecice measured 7x5x3 cm with covering skin ellipse measured 4.5x1 cm

• Tumor extent

o Number: An ill defined greyish white rubbery tumor bed side

o Size: 3x2x1 cm

o Skin involvement: Free grossly

• Distance from margins: 0.7 cm from superior margin (least margin)

• Lymph nodes number and size: 5 SLNs up to 1 cm

• Frozen section:

Free margins

Free SLNs(0/5)

Microscopic examination:

Total tumor regression in a previously diagnosed invasive carcinoma

• Lymph node involvement: Free SLNs(0/5)

• Resection Margins: Free margins

• Histologic response to therapy: Pathologic complete response

Diagnosis:

Right breast cancer , BCS & SLNs specimens:

-Pathologic complete response in a previously diagnosed invasive carcinoma

-Free margins

-Free SLNs(0/5)

-Stage : ypT0, ypN0

Verified by :

dr.esraa magdy

Printing Date : 19/06/2023 14:35:35

Page 1 of 2

Printing By : marwa moham

جمهورية مصر العربية
بطاقة تحقيق الشخصية

إيمان
محي الدين عبد الحميد الجمل
مصطفى كامل البحري - البر الشرقي
شبين الكوم - المنوفية

١٩٨٣/٠٩/٠١ ٢٨٣.٩.١١٧.٠٠٣٤٥

ID8897582

٢٠١٩/٠٩ ٢٨٣.٩.١١٧.٠٠٣٤٥

حاصله على ليسانس آداب
قسم علم نفس
أنثى
معلمة
متزوجة
محمد جلال عبد المجيد الجمسى
البطاقة سارية حتى ٢٠٢٩/٠٩/١٧

١٢